

PLABABLE

GEMS

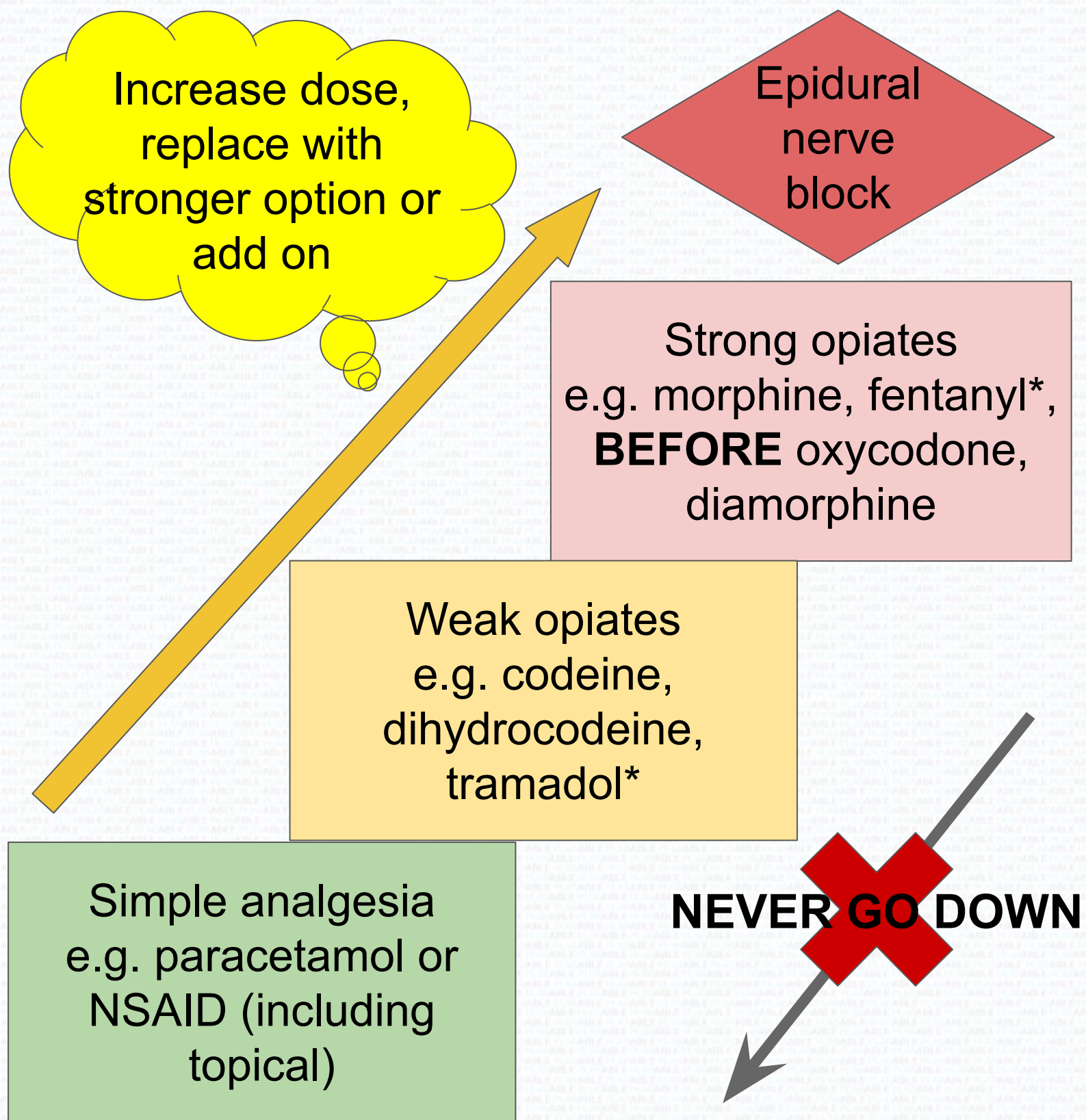
VERSION 1.1

ONCOLOGY



PLABABLE

Pain Ladder

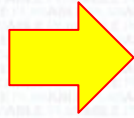


In pain caused by vertebral metastasis (e.g. breast), all 3 steps might be used at the same time as **lower vertebrae pain** can be very **severe**

*After an **open surgery**, use **morphine** as analgesic, can be weaned off after

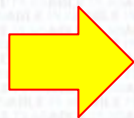
Oncology Complications

Antispasmodic
e.g. mebeverine



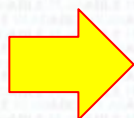
Visceral pain

Haloperidol



Intractable hiccup due
to metastasis

Senna



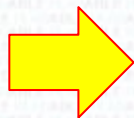
Constipation secondary
to opioids

Metoclopramide



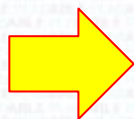
Vomiting due to opioids

Steroids



Severe anorexia

Cholestyramine



Itching due to jaundice

Nausea & Vomiting

Causes

Glioblastoma (intracranial tumour)

→ ↑ Intracranial pressure

Causing 🤢nausea and vomiting🤮

Treatment

First-line antiemetic for nausea & vomiting secondary to ↑ intracranial pressure is

Cyclizine

Dexamethasone

1. Shrink oedema around tumour
2. Reduce intracranial pressure
3. Relieve nausea & vomiting

Superior Vena Cava Obstruction

- It is an oncological emergency caused by compression of the SVC
- Most commonly associated with lung cancers

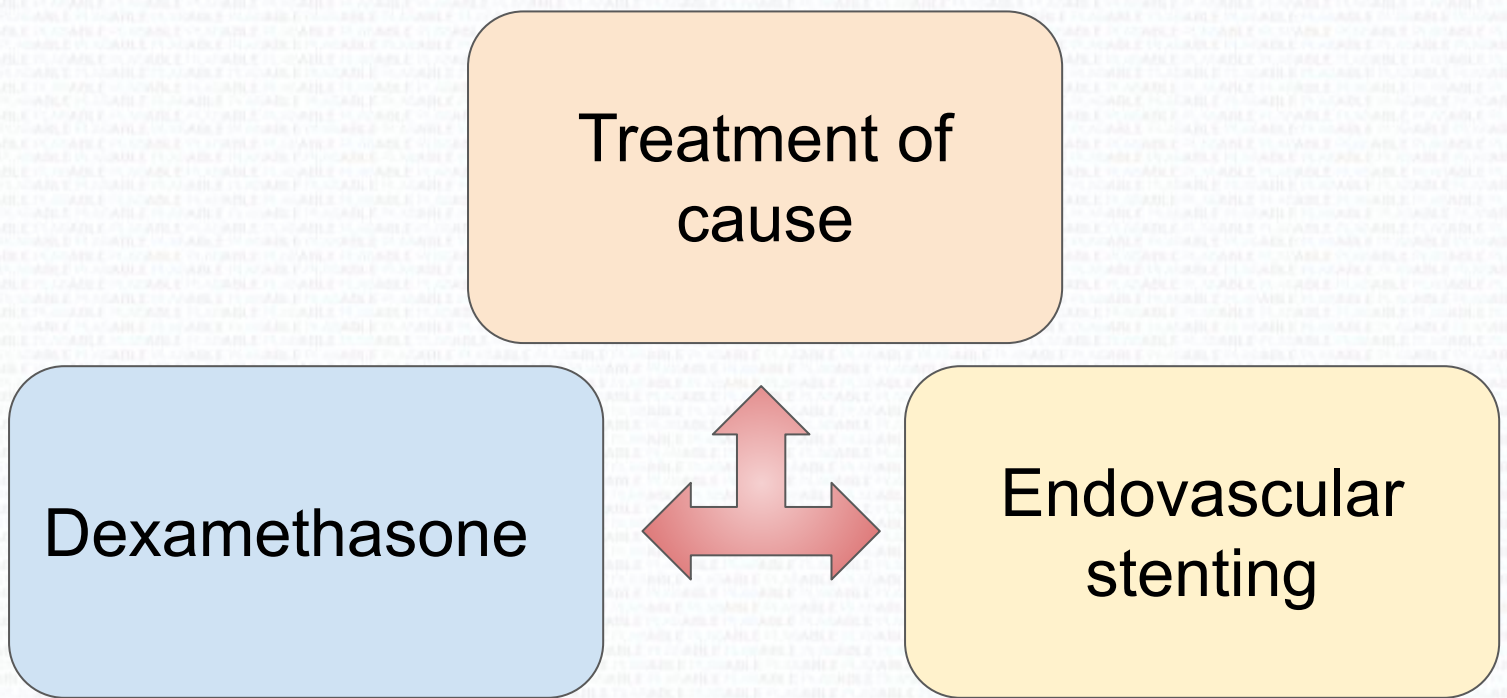
Features

- **Dyspnoea** - shortness of breath
- **Swelling of face, arms, and neck** - maybe conjunctival and periorbital oedema
- **Facial plethora**
- **Headache** → often worse in the morning
- Visual disturbance
- Pulseless jugular venous distention
(**distention of neck and upper chest veins**)

Causes

- Common malignancies e.g. small cell lung cancer, lymphoma
- Others metastatic seminoma, Kaposi's sarcoma, breast cancer, aortic aneurysm
- Mediastinal fibrosis
- Goitre
- SVC thrombosis

Superior Vena Cava Obstruction Management



Brain trainer:

Patient diagnosed with **small cell lung cancer**, presented with **shortness of breath** and **headache** for several weeks. He has **swelling of face, neck and arms**, and **distention of his neck and veins** which has been worsening over weeks. On examination, he has **facial plethora**, no audible stridor or laryngeal oedema

- Most likely **Dx: SVC obstruction**
- Immediate/next management: **dexamethasone**
- Treatment of choice: **endovascular stenting of obstructed SVC**

Tumour Lysis Syndrome

Facts:

- Mainly after initiation of chemotherapy
- Mainly in **all type of leukemia** and **lymphoma in particular Burkitt's lymphoma**
- This is due to lysis of tumour cells and released excessive amount of uric acid, potassium, phosphate into blood



Features:

4H's

- H**yper**uricemia (↑ Uric acid)
- H**yper**kalaemia (↑ Potassium)
- H**yper**phosphatemia (↑ Phosphate)
- H**y**po**cal**caemia (↓ Calcium)

High uric level can trigger
Gout!

Intestinal obstruction

Brain trainer:

A patient with metastatic colorectal cancer presents with severe abdominal colicky pain and vomiting of fecal content. CT scan shows diffuse intra-abdominal disease with ascites. On examination, his abdomen is distended with high-pitched bowel sounds. What is the most appropriate next management?

→ **Nasogastric tube**

Thyroid cancer

Brain trainer:

When a cancer patient is post thyroidectomy which laboratory parameter is most useful for screening?

→ **Thyroglobulin (Tg)**

After thyroidectomy blood thyroglobulin levels should be virtually undetectable

Biopsy results

Brain trainer:

A biopsy report for suspected testicular carcinoma shows macroscopically a greasy material which is comprised of hair and microscopically hyaline cartilage and mature squamous epithelium with keratinisation. What is the most likely diagnosis?

→ **Teratoma**

Diarrhoea

Brain trainer:

Which chemotherapy drug is used to treat a wide variety of cancers (including colon and breast) and might cause such a serious bout of diarrhoea that it may be necessary to stop?

→ **Capecitabine**

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